

Excelsa Health Westmoreland Hospital

532 W Pittsburgh St, Greensburg, PA 15601

DISCHARGE SUMMARY Document #: DIS-019

Patient & Admission

Name	Aaron Hayes
Date of Birth	1966-02-02
Phone	412-555-1104
Address	34123 Andrea Estate Butler, PA, 16001
MRN	MRN-471349
Admit Date	2026-05-04
Discharge Date	2026-05-10
Attending	Breanna Cantu, MD
Attending NPI	6159514846

Insurance

Primary Payer	Aetna
Primary Member #	W022941318

Primary Discharge Diagnosis

AVNRT s/p slow-pathway ablation

ICD-10: I48.3

Hospital Course

The patient presented with symptomatic atrioventricular nodal reentrant tachycardia (AVNRT) characterized by recurrent episodes of palpitations and presyncope refractory to rate-control medications. Electrophysiology study confirmed dual atrioventricular nodal physiology with inducible AVNRT at cycle lengths consistent with slow-pathway conduction. The patient underwent successful slow-pathway ablation via radiofrequency energy delivery. The procedure was uncomplicated, with elimination of dual nodal physiology and non-inducibility of AVNRT at the conclusion of the case. Post-ablation electrocardiogram showed normal sinus rhythm with intact AV conduction and no evidence of accelerated junctional rhythm.

During the six-day hospitalization, the patient remained hemodynamically stable with continuous telemetry monitoring. He experienced no recurrent arrhythmias and tolerated oral intake without difficulty. Troponin levels remained negative, and baseline renal function was preserved. The patient was maintained on aspirin for antiplatelet prophylaxis and continued his home antihypertensive regimen. Echocardiography was deferred given the absence of structural heart disease on prior imaging and normal left ventricular function. Patient education regarding activity restrictions, wound care, and recognition of warning signs was provided.

The patient is being discharged in stable condition with resolution of his presenting arrhythmia. He will follow

up with electrophysiology in two weeks for post-ablation assessment and rhythm surveillance. His primary care physician will coordinate ongoing management of hypertension and cardiovascular risk factors. The patient was counseled to avoid strenuous activity for one week and to report any recurrent palpitations, syncope, or chest discomfort immediately. Given the successful ablation and elimination of inducible AVNRT, long-term antiarrhythmic therapy is not anticipated.

Procedures Performed

Slow-pathway ablation for AVNRT	CPT 93653
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Medication Changes

Apixaban 5 mg BID	STARTED
Flecainide 100 mg BID	STARTED
Metoprolol tartrate 25 mg BID	ADJUSTED
Amiodarone	STOPPED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	26 days
Urgency Tier	ROUTINE
Clinical Flags	post-ablation anticoagulation required

Dictated and electronically signed by:
Breanna Cantu, MD
NPI: 6159514846
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